

Juvenile primary fibromyalgia syndrome.

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Juvenile primary fibromyalgia syndrome (JPFS) is a common musculoskeletal pain syndrome of unknown etiology characterized by widespread persistent pain, sleep disturbance, fatigue, and the presence of multiple discrete tender points on physical examination. Other associated symptoms include chronic anxiety or tension, chronic headaches, subjective soft tissue swelling, and pain modulated by physical activity, weather, and anxiety or stress. Research and clinical observations suggest that JPFS may have a chronic course that impacts the functional status and psychosocial development of children and adolescents. In addition, several factors have been implicated in the etiology and maintenance of JPFS including genetic and anatomic factors, disordered sleep, psychological distress, and familial and environmental influences. A multidisciplinary approach to treatment of JPFS is advocated, including pharmacologic and nonpharmacologic interventions (eg, psychotherapy, aerobic exercise, sleep hygiene).

Autonomic nervous system derangement in fibromyalgia syndrome and related disorders.

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Fibromyalgia syndrome is a chronic, painful musculoskeletal disorder of unknown etiology and/or pathophysiology. During the last decade many studies have suggested autonomic nervous system involvement in this syndrome, although contradictory results have been reported. This review focuses on studies of the autonomic nervous system in fibromyalgia syndrome and related disorders, such as chronic fatigue syndrome and irritable bowel syndrome on the one hand and anxiety disorder on the other, and highlights techniques of dynamic assessment of heart rate variability. It raises the potentially important prognostic implications of protracted autonomic dysfunction in patient populations with fibromyalgia and related disorders, especially for cardiovascular morbidity and mortality.

Newer treatments for fibromyalgia syndrome.

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Fibromyalgia syndrome is a common chronic pain disorder of unknown etiology. The lack of understanding of the pathophysiology of fibromyalgia has made this condition frustrating for patients and clinicians alike. The most common symptoms of this disorder are chronic widespread pain, fatigue, sleep disturbances, difficulty with memory, and morning stiffness. Emerging evidence points towards augmented pain processing within the central nervous system (CNS) as having a primary role in the pathophysiology of this disorder. Currently the two drugs that are approved by the United States Food and Drug Administration (FDA) for the management of fibromyalgia are pregabalin and duloxetine. Newer data suggests that milnacipran, a dual norepinephrine and serotonin reuptake inhibitor, may be promising for the treatment of fibromyalgia. A double-blind, placebo-controlled trial of milnacipran in 125 fibromyalgia patients showed significant improvements relative to placebo. Milnacipran given either once or twice daily at doses up to 200 mg/day was generally well tolerated and yielded significant improvements relative to placebo on measures of pain, patient's global impression of change in their

disease state, physical function, and fatigue. Future studies are needed to validate the efficacy of milnacipran in fibromyalgia.

Symptoms and signs in fibromyalgia syndrome.

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Fibromyalgia syndrome (FM) is a common chronic pain condition that affects at least 2% of the adult population. Chronic widespread pain is the defining feature of FM, but patients may also exhibit a range of other symptoms, including sleep disturbance, fatigue, irritable bowel syndrome, headaches, and mood disorders. The etiology of FM is not completely understood and the syndrome is influenced by factors such as stress, medical illness, and a variety of pain conditions. Establishing diagnosis may be difficult because of the multifaceted nature of the syndrome and overlap with other chronically painful conditions. A unifying hypothesis is that FM results from sensitization of the central nervous system; this new concept could justify the variety of characteristics of the syndrome. FM symptoms can be musculoskeletal, non-musculoskeletal, or a combination of both; and many patients will also experience a host of associated symptoms or conditions. The ACR classification criteria focus only on pain and disregard other important symptoms; but three key features, pain, fatigue and sleep disturbance, are present in virtually every patient with FM. Several other associated syndromes, including circulatory, nervous, digestive, urinary and reproductive systems are probably a part of the so called central sensitivity or sensitization syndrome. A minority subgroup of patients (30-40%) has a significant psychological disturbance. Psychological factors are an important determinant of any type of pain, and psychological comorbidity is frequent in FM. Psychiatric disorders most commonly described are mood disorders, but psychiatric illness is not a necessary factor in the etiopathogenesis of FM.

Neuronutritional Approach to Fibromyalgia Management: A Narrative Review.

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Fibromyalgia (FM) is a complex and common syndrome characterized by chronic widespread pain, fatigue, sleep disturbances, and various functional symptoms without clear structural or pathological causes. Affecting approximately 1-5% of the global population, with a higher prevalence in women, FM significantly impacts patients' quality of life, often leading to considerable healthcare costs and loss of productivity. Despite its prevalence, the etiology of FM remains elusive, with genetic, environmental, and psychological factors, including nutrition, being implicated. Currently, no universally accepted treatment guidelines exist, and management strategies are often symptomatic. This narrative review explores the potential of a neuronutritional approach to FM management. It synthesizes existing research on the relationship between FM and nutrition, suggesting that dietary interventions could be a promising complementary treatment strategy. Various nutritional interventions, including vitamin D, magnesium, iron, and probiotics supplementation, have shown potential in reducing FM symptoms, such as chronic pain, anxiety, depression, cognitive dysfunction, sleep disturbances, and gastrointestinal issues. Additionally, weight loss has been associated with reduced inflammation and improved quality of life in FM patients. The review highlights the anti-inflammatory benefits of plant-based diets and the low-FODMAPs diet, which have shown promise in managing FM symptoms and related gastrointestinal disorders. Supplements such as vitamin D, magnesium, vitamin B12, coenzyme Q10, probiotics, omega-3 fatty acids, melatonin, S-adenosylmethionine, and acetyl-L-carnitine are discussed for their potential

benefits in FM management through various mechanisms, including anti-inflammatory effects, modulation of neurotransmitters, and improvement of mitochondrial function. In conclusion, this review underscores the importance of considering neuronutrition as a holistic approach to FM treatment, advocating for further research and clinical trials to establish comprehensive dietary guidelines and to optimize management strategies for FM patients.

Fibromyalgia: a critical digest of the recent literature.

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Fibromyalgia (FM) syndrome is a chronic disease with unknown etiology, characterised by widespread pain and fatigue. Moreover, several patients manifest non-specific symptoms such as sleep disturbances, mood disorders, and neurocognitive impairment. We have reviewed the literature of the past year to underline the progress of research in the fields of etiopathogenesis, therapies and assessment, considering articles published between November 2012 and November 2013.

Association Between Systemic Neuroinflammation, Pain Perception and Clinical Status in Fibromyalgia Patients: Cross-Sectional Study.

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Fibromyalgia (FM) is characterized by chronic pain and a complex array of symptoms, with neuroinflammation implicated in its pathophysiology. This study aimed to explore the association between neuroinflammation, measured through interleukin levels (IL-1, IL-6, IL-8), and clinical outcomes in FM patients. Using a cross-sectional study design, blood levels of these interleukins were correlated with pain severity and disability, assessed via the Fibromyalgia Impact Questionnaire (FIQ) and pain measures. Results indicated that IL-6 and IL-8 may particularly serve as biomarkers for pain severity and disability in FM patients, showing significant associations with worse clinical outcomes. Elevated IL-8 levels, for instance, correlated strongly with increased pain perception and higher disability scores. These findings suggest that specific interleukins are not only elevated in FM but are actively involved in the modulation of pain and disability, underscoring the role of systemic neuroinflammation in the clinical severity of FM. This study contributes to a deeper understanding of the inflammatory mechanisms in FM and underscores the potential of targeting interleukins in therapeutic strategies.

[Fibromyalgia and psychiatry: 35 years later... what's new?].

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Fibromyalgia (FM) is a complex disorder that affects 2 to 5% of the general population worldwide at any age and any sex, but more frequently in adult women. The variability of symptoms and the frequency of comorbidities among patients with fibromyalgia make this a difficult disorder to diagnose and treat. New diagnostic criteria are available to improve the diagnosis and care of patients. We propose the first French translation of the new diagnostic criteria proposed by the American College of Rheumatology (ACR) 2010. Although the etiology of fibromyalgia remains unclear, evidences suggest that biologic,

genetic and environmental factors are involved. This chronic psychophysical suffering state of fibromyalgia adversely affects the patient's quality of life, performance and mood. Studies report a higher prevalence of psychiatric disorders in patients with fibromyalgia that requires careful history and evaluation for the presence of primary or secondary mood disturbances and psychological stress. Despite the absence of strong evidence of the fibromyalgia pathogenesis, evidence base medicine lead us to consider a biopsychosocial model and a multidisciplinary treatment. Treatment of fibromyalgia requires a comprehensive and multidimensional approach with patient education, cognitive behavior therapy, exercise, physical therapy and pharmacological therapy. Today, the serotonin noradrenaline reuptake inhibitors (milnacipran and duloxetine) and the antiepileptic (pregabalin) are the most effective drugs available for the treatment for fibromyalgia. Therapeutic and pathophysiologic researches seem necessary in this disease and should establish an exemplary model for integrative disease where somatic and psychic form a continuum.

Fascial preadipocytes: another missing piece of the puzzle to understand fibromyalgia?

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Fibromyalgia (FM) syndrome is a chronic condition causing pain, affecting approximately 0.5%-6% of the developed countries' population, and on average, 2% of the worldwide population. Despite the large amount of scientific literature available, the FM etiology is still uncertain. The diagnosis is based on the clinical presentation and the severity of the symptomatology. Several studies pointed out pathological alterations within the central nervous system, suggesting that FM could originate from a central sensitization of the pain processing centers. Research supports the thesis of a peripheral neuropathic component, with the finding of axonal damages. The fibromyalgia patient has many myofascial system abnormalities, such as pain and fatigue, impairing the symptomatic profile. This paper revises the myopathic compensations, highlighting the possible role of the fascia in generating symptoms, being aware of the new information about the fascia's activity in stimulating inflammation and fat cell production.

[Fibromyalgia and its diagnosis].

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Fibromyalgia or fibromyalgic syndrome (FMS) is a clinical picture of unknown origin that is characterized by generalized, incapacitating and chronic pain together with the demonstration in the physical examination of previously defined points in which moderate pressure causes pain, called fibromyalgia tender points (FTP). FMS lacks objective, analytic imaging or pathological data, so that its diagnosis is based exclusively on subjective data, such as that the pain reported by the patient and the pain caused by the FTP pressure. Although the fibromyalgia classification criteria of the American College of Rheumatology are not diagnostic criteria, they have been extensively used to diagnose FMS in patients with chronic diffuse arthromyalgias. Fibromyalgia diagnosis reduces the patient's anxiety, avoiding complementary expensive and unnecessary tests and it allows the patient to share his/her fears, illnesses and expectations with other human beings who suffer the same problem.
